AARAMBH

Follow-up Navigator

Care That Fits Real Life

ROUND 1 | CASE INVESTIGATION & PROBLEM-SOLUTION BRIEF

Case: The Follow-Up Blind Spot • Healthcare

Executive Summary

A district hospital can issue a follow-up plan without knowing whether the patient understood it, could be reached safely, was able to travel, or actually completed the next step. The case pack describes a high-volume setting in which record quality, shared or unreachable phone numbers, long travel distances, waiting time, and limited staff capacity can make follow-up difficult to coordinate. A "missed" appointment may also reflect incomplete documentation rather than a patient who did not attend.

Aarambh Follow-up Navigator is a proposed, human-supervised workflow that links clinician-approved instructions, patient understanding, barrier reporting, practical visit planning, staff action, and verified outcomes. Its core is not another reminder app: it is a way to find unresolved follow-up needs and help people complete the next step. A visit-planning module can display a waiting-time estimate and departure guidance only when dependable queue, appointment, and travel-time inputs are available.

Selected Problem Statement

PROBLEM: Patients and hospital teams lack one reliable, accessible workflow to turn clinician-approved follow-up plans into understood and achievable next steps, identify barriers early, plan visits with realistic timing information, and verify whether care was completed—without confusing poor records with patient non-attendance or automating clinical decisions.

How Might We…?

How might we help patients understand and plan their next care step—and help hospital staff verify and resolve follow-up gaps—through accessible guidance, consent-aware barrier checks, carefully qualified visit-time estimates, and human-reviewed coordination?

The Proposed Care Journey

CORE FLOW: Clinician-approved plan → Clear explanation → Patient understanding check → Barrier and contact check → Visit planning (when data are reliable) → Staff action → Follow-up status → Verified outcome

1. MAP — Understand the Case Before Designing

Situation

The challenge describes a simulated high-volume district-hospital clinic. Clinicians can identify that follow-up is needed, but the system struggles to ensure patients return within the recommended time. Contact data may be shared or unreachable, travel may be difficult, and staff cannot manually call or prioritise every record. The boundary is explicit: this is a workflow and continuity-of-care challenge, not a diagnostic or emergency-triage challenge.

Case-Pack Signals

Signal in the case pack	Why it matters	What remains unknown
About 900 outpatient visits per day (simulated clinic)	Manual review of every follow-up may not scale.	Actual arrival patterns, clinic-level volumes, staffing, and peak-hour variation.
28% of patients marked for follow-up in a three-month sample did not return within the recommended window	A potentially important follow-up gap exists.	Which cases remain clinically important, were rescheduled, attended elsewhere, or were recorded incorrectly.
19% of records use a shared household phone number; 7% have an unreachable number	A message may reach the wrong person or no one.	Consent, preferred contact channel, phone ownership, and safe alternatives.
Instructions are stored as free text and vary by clinician	Meaning and next steps may be ambiguous or difficult to explain consistently.	Which fields are reliably captured and what language versions are approved.
A small records team cannot manually prioritise every missed appointment	Unresolved cases can remain unnoticed.	What workload is feasible and which staff actions can be supported safely.
Clinical teams reject autonomous diagnosis, treatment changes, or emergency triage	The system must remain within a clear support role.	Exact escalation and approval procedures at the hospital.

Stakeholders and Their Needs

Stakeholder	Primary need / responsibility
Patients	Understand the next step, know when and where to attend, access help through an appropriate channel, and retain control over contact preferences.
Family members / caregivers	Support the patient only where authorised; a shared phone must not be treated as automatic permission to disclose sensitive information.
Clinicians / clinical team	Create and approve clinical instructions, clarify ambiguity, and retain authority over medical decisions and clinical urgency.
Registration and records staff	Maintain accurate attendance and contact status, reconcile records, and see a manageable queue of cases requiring action.
Nurses / community health workers	Help patients who need an explanation, a different channel, or practical coordination.
Hospital administration	Set policy, approve data use, monitor operational metrics, and decide whether the workflow is feasible.

Non-Negotiable Constraints

- No diagnosis, prescribing, treatment modification, autonomous urgency ranking, or emergency triage.
- Patient privacy, consent-aware contact, data minimisation, and role-based staff access.
- Support multilingual and non-digital pathways; an app must not be the only route to care.
- Missing, contradictory, stale, or unverified information must be flagged—not silently guessed.
- A message sent, call attempted, or risk score generated is not proof of understanding or completed care.

2. DIVERGE — Compare Plausible Problem Framings

Before choosing a solution, the team considered multiple ways to frame the challenge. These options are deliberately distinct; none is assumed correct until investigated against the case evidence and research.

Problem framing	Potential value	Blind spot / constraint
A. Instruction clarity: patients do not always understand the clinician-approved next step.	Focused and testable; supports accessible communication and a direct understanding check.	By itself, it does not resolve record errors, travel barriers, waiting-time uncertainty, or staff ownership of follow-up.
B. Missed-appointment risk: identify records that may need proactive follow-up.	Could help allocate limited staff attention when reliable local data and a useful intervention are available.	Can mislabel patients when records are incomplete; requires clear labels, local validation, and a response beyond producing a score.
C. End-to-end follow-up and visit planning: help patients understand, plan, access, and complete the next care step.	Can connect patient understanding, access/contact barriers, record reconciliation, staff action, and outcome verification.	Broader scope; must be staged carefully, and any waiting-time/departure estimate depends on dependable input data.

3. INVESTIGATE — Evidence, Barriers, and Uncertainty

What the Case Evidence Supports

The case pack points to an interconnected problem involving information quality, access, contact reliability, workflow capacity, and follow-up verification. These figures are presented by the challenge as simulated case evidence; they should not be represented as audited statistics from a real hospital.

Research Used to Refine the Problem

Source / finding	Implication for Aarambh	Caution
Das et al. studied missed follow-up appointments among patients with hypertension at public health facilities in Punjab, India. Among analysed reasons, distance from home was 22.2%, lack of instructions/guidance or reminders 15.2%, and long waiting hours 7.0% [1].	Travel, clarity, and waiting experience should be investigated alongside communication. The system should support practical planning, not only send reminders.	The study focused on hypertension follow-up in Punjab; the percentages must not be assumed to apply to all patients or hospitals.
The same study found that 49 patients had been wrongly classified as missed visits because facility records did not capture their attendance [1].	Record reconciliation is a core safety feature. Before outreach or risk labelling, check for duplicate, delayed, or incomplete attendance records.	This is a finding from that study, not a measured rate for the simulated clinic in the hackathon case.
A systematic review of 24 studies examined machine-learning approaches to appointment no-shows. It reported recurring associations with age, previous missed appointments, and appointment lead time; decision trees were frequently identified as suitable models [2].	A transparent prediction component may be considered later, using local data and supportive interventions. A simple baseline should be compared before selecting a model.	The review reports heterogeneous studies, and real-world validation was limited. Predictive accuracy alone does not prove better follow-up outcomes.
The case pack notes shared or unreachable phone numbers and variable free-text instructions.	Build contact preference, safe-message rules, instruction provenance, and human escalation into the workflow.	The case pack does not establish which alternate channels patients will accept or which language translations are approved.

Barrier Map

Barrier category	Examples to investigate	Design response to test
Understanding	Complex terminology, unclear date/time, language mismatch.	Plain-language, clinician-grounded instructions plus teach-back confirmation.
Access and visit planning	Travel distance, cost or mobility constraints, uncertain queue duration.	Patient-reported barriers; check-in guidance; qualified waiting-time and travel estimates when data exist.
Contact and privacy	Shared phone, invalid number, switched-off phone, no consent for a caregiver contact.	Verify preferred channel and permission; use neutral messages until identity/contact rules are satisfied.
Record quality	Attendance not entered, duplicate records, conflicting dates or status.	Reconcile before classifying; keep status as “unverified” when evidence conflicts.
Staff capacity	Large queue of follow-up cases; unclear ownership for next action.	Transparent task queue, reason for each flag, named owner, and status tracking.

Working Hypotheses (To Be Tested, Not Assumed)

- Patients may be more able to complete follow-up when instructions are understandable and the next action is confirmed rather than merely delivered.
- Some apparent non-attendance may be due to record or contact-data gaps; reconciliation before outreach may reduce avoidable false alerts.
- Waiting-time information may help visit planning only if it is timely, calibrated, and displayed separately from the required appointment/check-in time.
- A rules-based, staff-reviewed workflow may be safer and more feasible for an early prototype than an unvalidated no-show prediction model.

4. DECIDE — Selected Problem Definition

SELECTED FRAMING: Care That Fits Real Life

An AI-powered care navigator system that predicts delayed follow-up risks, uncovers patient-specific barriers, and helps hospitals turn follow-up plans into achievable care journeys.

Final How Might We Statement

How might we help patients understand and plan their next care step—and help hospital staff identify and resolve follow-up gaps—using accessible clinician-approved guidance, consent-aware communication, reliable visit-time estimates where possible, and human-reviewed coordination?

Why This Is the Best Round 1 Direction

- It is grounded in the case pack's signals: high volume, missed follow-up status, shared/unreachable contacts, free-text instructions, and limited staff capacity.
- It directly addresses research-supported barriers such as distance, unclear guidance, long waits, and record misclassification [1].
- It avoids presenting prediction as the intervention. Any risk flag must lead to a useful, proportionate staff action and later outcome verification.
- It is testable using a prototype with de-identified or synthetic records, then a supervised pilot only after appropriate approval.

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5. DESIGN — Aarambh Follow-up Navigator

Aarambh is a proposed AI-assisted coordination layer for patients, clinicians, and hospital staff. It should connect to existing workflow where possible rather than assume the hospital can immediately replace its registration or records system.

End-to-End Workflow

WORKFLOW: Clinician approves follow-up plan → Required fields validated → Patient receives accessible explanation → Patient confirms/asks for help → Contact and access barriers captured → Visit plan shown (where data are reliable) → Staff task created if needed → Record and follow-up status reconciled → Outcome verified

Core Modules

Module	What it does	Human safeguard / output
1. Follow-up plan capture	Reads or receives approved next-step instructions, appointment window, location, and responsible service.	Missing or conflicting required fields are marked "needs clarification"; AI does not invent values.
2. Accessible explanation	Converts approved text into shorter, patient-friendly, multilingual wording using a controlled prompt/template and approved glossary.	Output is labelled AI-generated, linked to the source instruction, and routed for clinician/staff review where policy requires.
3. Understanding check	Uses teach-back, a simple confirmation, or a request-for-help option.	"Not sure," no response, or contradictory responses create an assistance task—not a pass.
4. Barrier and contact check	Records preferred language/channel, consent, shared-phone caution, travel/access issue, or contact failure.	Only necessary details are collected. The system distinguishes "unable to reach" from "refused" or "not attended."

Module	What it does	Human safeguard / output
5. Visit planner	Shows scheduled time, check-in rule, estimated travel time, and a predicted waiting range when supported by reliable data.	Scheduled appointment rules take precedence; forecast has a timestamp and uncertainty. If confidence/data are inadequate, show “estimate unavailable.”
6. Staff coordination queue	Groups records needing clarification, contact verification, barrier support, or outcome review.	Each item shows why it was flagged, confidence/status, due window, and the next suggested administrative action.
7. Outcome verification	Captures attendance or another approved follow-up outcome from an authoritative record or staff confirmation.	A sent reminder or attempted call is never counted as completion.

Design Principles

- Patient-centred: optimise for a clear, achievable next step, not message volume.
- Transparent: show the source instruction, status, reason for a flag, and when any timing estimate was updated.
- Low burden: prioritise tasks staff can act on; avoid repeated alerts without a meaningful next action.
- Accessible: support local languages, voice/IVR or staff-assisted routes, and non-digital users.
- Reversible: allow staff to correct a record, dismiss a false flag with a reason, and audit changes.

6. BASIC SOFTWARE REQUIREMENTS SPECIFICATION (SRS)

Users and Permissions

Patient / Authorised Caregiver: The patient or authorised caregiver can view the patient-facing next-step explanation, confirm understanding, request help, share optional access constraints, and choose an appropriate contact route. They are not allowed to view unrelated records or change clinician-approved instructions.
Clinician / Authorised Clinical Staff: Clinicians or authorised clinical staff can enter or approve medical instructions, clarify ambiguity, review explanations under local policy, and make all medical decisions. They are not allowed to delegate medical judgement to the AI.
Registration / Follow-up Staff: Registration and follow-up staff can reconcile attendance and contact details, assign administrative tasks, record follow-up attempts, and verify status using approved sources. They are not allowed to treat a model-generated score as proof of non-attendance or clinical urgency.
Hospital Administrator: Hospital administrators can configure permissions, templates, retention policies, queue definitions, and performance reporting. They are not allowed to access identifiable patient data without a valid role and purpose.

Expected Inputs and Outputs

Inputs: The system takes approved next-step instructions, due dates or time windows, clinic or location details, appointment or check-in rules, patient identifiers, preferred language and contact route, relevant consent flags, attendance and follow-up status, available queue timestamps, and optional travel-time information.

Outputs: The system provides a patient-friendly explanation linked to the approved source, records comprehension and barrier/contact status, generates a visit plan with clear estimate provenance, assigns staff tasks along with the reason for each task, and maintains follow-up status, verified outcomes, and an auditable record of the care journey.

Functional Requirements

ID	Requirement	Priority / verification
FR-01	Validate required follow-up fields and flag missing, stale, contradictory, or malformed data.	Must / missing-data test cases.
FR-02	Keep the original clinician-approved instruction unchanged and preserve its source/version.	Must / source comparison.
FR-03	Produce a clearly labelled, plain-language explanation in a supported language without adding clinical content.	Must / clinician-reviewed test set.
FR-04	Record understanding as confirmed, unclear, requested assistance, or unknown; do not infer confirmation from message delivery.	Must / scripted user tests.
FR-05	Capture contact preference/consent and practical barriers; distinguish contact failure from non-attendance.	Must / shared-phone scenarios.
FR-06	Offer staff a traceable list of unresolved cases, each with a reason, status, and next administrative action.	Must / end-to-end workflow test.

ID	Requirement	Priority / verification
FR-07	Display waiting-time and departure guidance only when the underlying appointment/queue/travel inputs meet configured validity rules.	Conditional / test reliable and unreliable data.
FR-08	Record outreach attempts, corrections, staff actions, attendance checks, and verified outcomes with timestamp and role.	Must / audit-log test.
FR-09	Support staff-assisted and non-digital access paths.	Must / accessibility walkthrough.
FR-10	Never diagnose, prescribe, modify treatment, autonomously triage, or override a clinician.	Must / adversarial prompt and workflow tests.

Non-Functional Requirements

- Privacy and security: role-based access, secure transmission/storage, minimum necessary data, retention limits, and auditable access.
- Reliability: fail safely; when a data feed or estimate is unavailable, display that limitation and retain the confirmed appointment instructions.
- Usability: readable language, screen-reader compatible structure, low-bandwidth operation where feasible, and assisted alternatives.
- Explainability: every staff flag must show its trigger; every patient explanation must preserve source provenance.
- Maintainability: templates, supported languages, thresholds, and queue rules are versioned and change-controlled.

7. TECHNICAL APPROACH, VISIT PLANNING, AND HUMAN CONTROL

Proposed Technical Architecture

DATA FLOW: Hospital appointment / records export + clinician-approved plan + patient-stated preferences → Input validation and record reconciliation → Rules/template layer → Optional language model for grounded rewriting → Comprehension and barrier capture → Staff task queue → Outcome update and audit log

For the Round 1 prototype, use synthetic or properly de-identified records if direct hospital integration and approvals are not available. A transparent rules-based baseline is preferable to claiming a trained prediction model without adequate local data. Model-based prioritisation can be tested later only with an agreed label definition, data-quality review, held-out evaluation, and a human action protocol.

Waiting-Time and Departure Logic

Scenario	Proposed behaviour	Safety rule
Scheduled appointment	Calculate a suggested departure time from appointment time, estimated travel duration, and the hospital's check-in buffer. Display expected waiting time separately.	Do not subtract the predicted queue delay from the required arrival time or tell patients to arrive late.
Walk-in / flexible arrival window	If the hospital explicitly allows flexible arrival and a validated queue forecast exists, show a suggested service window and a departure estimate with update time and uncertainty.	Never show a false precision. If inputs are stale or validation fails, suppress the forecast.
Missing traffic/travel input or unavailable queue feed	Use the confirmed appointment/check-in instructions and allow patient/staff to enter a travel estimate manually, if appropriate.	Show "estimate unavailable"; do not imply live tracking or guaranteed waiting time.

Simple scheduled-visit rule: suggested departure time = appointment time − estimated travel duration − required check-in buffer. Waiting-time estimates are a separate expectation-setting aid. The hospital's official instructions always take priority.

Human–AI Responsibility Split

AI may assist with: The AI may assist in rewriting approved instructions into plain, patient-friendly language, detecting missing information, suggesting administrative follow-up tasks, summarising declared barriers, and estimating queue duration when validated data are available.

A human must retain control of: A human must retain control over approving clinical instructions, resolving ambiguous or conflicting medical information, determining clinical urgency, making decisions related to diagnosis, treatment, or prescribing, handling emergency responses, making the final judgement about clinical significance, and authorising contact or disclosure where required.

Illustrative Staff Dashboard (Text Wireframe)

Queue view	Record card should show
NEEDS CLARIFICATION	Instruction field missing/conflicting; source; owner; next review action.
CONTACT / ACCESS BARRIER	Preferred channel/language; consent state; contact attempt history; barrier stated by patient; safe next action.
FOLLOW-UP DUE / UNVERIFIED	Due window; attendance status source; last updated time; whether record reconciliation is pending.
VISIT PLAN	Appointment/check-in time; estimated travel duration and source; wait estimate (if valid), last update, and uncertainty.
COMPLETED / VERIFIED	Evidence source and timestamp; staff role; resolved status; correction history if any.

Implementation Stages

1. **Case Validation:** The first stage involves mapping the local workflow, interviewing patients and staff where permitted, defining terms such as “missed,” “due,” “unverified,” and “completed,” and auditing the available data fields to ensure that the system is based on realistic operational requirements.

2. **Clickable / Functional Prototype:** The prototype will use synthetic data to demonstrate the instruction source, patient understanding status, barrier capture, staff follow-up queue, and audit trail. A waiting-time view will be developed using simulated and clearly labelled data when live data are unavailable.

3. **Offline Validation:** The system will be tested for language fidelity, handling of missing or conflicting records, contact privacy, queue-time estimation against a simple historical baseline, and overall usability for hospital staff.

4. **Supervised Pilot Proposal:** Before any real-patient use, the project will seek institutional approval, privacy and security review, clinician and staff oversight, an appropriate consent workflow, and defined outcome monitoring to ensure safe and responsible deployment.

8. VERIFY — Metrics and Acceptance Tests

A reliable evaluation begins with a baseline. Results should be separated by supported language, contact channel, digital/non-digital route, and other operationally relevant groups where sample sizes and privacy rules permit. Targets should be agreed with the hospital and not invented from the simulated case pack.

Key Evaluation Metrics

Metric	Definition / measurement	Why it matters
Understanding rate	Patients who correctly identify the next required step ÷ patients assessed, using a defined teach-back or equivalent check.	Measures understanding, not message delivery.
Follow-up completion rate	Required follow-ups verified as completed within the approved window ÷ follow-ups due, after record reconciliation.	Measures the outcome the system is meant to support.
Barrier detection usefulness	Agreement between system-recorded barriers and patient/staff-confirmed barriers; report misses as well as false positives.	Prevents treating a guessed barrier as fact.
False alert rate	Flags closed as unnecessary after review ÷ all reviewed flags.	Measures avoidable staff workload and misclassification.
Record correction / reconciliation rate	Records corrected or resolved after a suspected status/attendance mismatch.	Tracks the “false missed visit” risk.
Waiting-time prediction error	Compare predicted wait with actual wait on held-out sessions using mean absolute error and error by time of day/clinic.	Checks whether estimates are useful and honest.
Staff response time	Time from unresolved case detection to first meaningful staff action.	Shows whether alerts improve coordination.
Accessibility and completion by channel	Task completion and comprehension through app, phone/voice, and staff-assisted routes.	Checks that non-digital users are not excluded.
Privacy / safety events	Count and investigate unauthorised disclosures, incorrect instruction changes, or actions outside scope.	Safety is a release gate, not just a performance metric.

Acceptance Tests for the Prototype

Test	Pass condition
AT-01: missing date or conflicting follow-up instruction	The system flags the case for clarification and does not generate an assumed date or medical instruction.
AT-02: AI explanation fidelity	Every critical action, date, dose or other clinician-specified detail (if present) matches the approved source; output is visibly labelled AI-generated and linked to the source.
AT-03: patient does not understand	The record remains unresolved and creates a help/escalation path; message delivery is not treated as confirmation.
AT-04: shared household phone	No sensitive details are disclosed to an unverified recipient; the system follows configured consent and identity rules or uses a neutral notification.
AT-05: attendance status is contradictory	The case is marked unverified and routed for record reconciliation before no-show outreach or a risk label.
AT-06: unreachable contact	The system records the attempt, avoids endless retries, and offers an approved alternate or staff-assisted path.
AT-07: scheduled visit timing	Departure guidance honours the appointment time and check-in buffer; predicted wait is displayed separately and never used to recommend late arrival.
AT-08: unreliable queue data	The wait estimate is suppressed or clearly unavailable; no unsupported “real-time” claim is shown.
AT-09: model / rules validation	Compare any proposed predictor with a simple baseline on held-out sessions; do not enable patient-facing prediction if it is not demonstrably useful or calibrated for the intended context.
AT-10: prohibited clinical request	The system does not diagnose, prescribe, alter treatment, or make an emergency-triage decision; ambiguous medical content is escalated.

9. DOCUMENT — Auditability, Privacy, and Failure Handling

Minimum Care-Journey Record

TRACEABLE RECORD: Clinician instruction and version → AI explanation and language → Patient confirmation / uncertainty → Consent and contact route → Barrier reported / source → Staff action and owner → Appointment and attendance status → Follow-up status → Verified outcome and evidence source

Required Documentation Rules

- Keep original clinician-approved content distinct from generated explanation; retain the source/version and identify the generated text as AI-assisted.
- Log important events with timestamp, actor/role, action, source status, and any correction reason. Restrict access to authorised users and the minimum data needed for each task.
- Retain uncertainty explicitly: “unknown”, “contact failed”, “patient requested help”, “attendance unverified”, and “not completed” must not be collapsed into one status.
- A patient or staff correction should be traceable; corrections must not silently overwrite the previous history.
- Use approved retention/deletion policies, secure transfer/storage, and audit access. Obtain institutional privacy/security review before any real patient data are processed.

Failure Modes and Mitigations

Failure mode	Potential harm	Mitigation / safe response
Wrong or incomplete source instruction	Patient receives inaccurate next-step guidance.	Block generation for critical missing/conflicting fields; show source; require authorised clarification.
Shared phone / wrong recipient	Private health details reach someone without permission.	Verify consent and recipient; use neutral content; provide a non-digital/staff-assisted alternative.
Attendance not recorded promptly	Attended patient is wrongly flagged as absent.	Use “unverified” state, reconcile records, and show data freshness before outreach.
False barrier / risk label	Staff time is wasted or a patient is unfairly labelled.	Explain trigger; permit correction/dismissal with reason; do not penalise or deprioritise care.
Inaccurate queue estimate	Patient arrives late or expects a guaranteed wait duration.	Show timestamp and uncertainty; respect appointment/check-in rules; fall back to official instructions.
Model or data feed unavailable	Workflow hides an unresolved case or shows stale prediction.	Fail visibly; preserve manual workflow; show last update and create a technical review task where appropriate.
Staff overload from alerts	Important tasks get buried.	Tune thresholds with staff; batch low-priority tasks; measure false alert rate and response time.

Assumptions and Questions to Resolve Before a Real Pilot

- Which source is authoritative for attendance, appointments, and follow-up completion? Who may correct each field?
- What is the approved process for consent, caregiver contact, shared phones, and sensitive notifications?
- Which languages and clinical glossaries are validated, and who approves revised translations or templates?
- Can appointment timestamps, check-in/queue timestamps, clinic capacity, and reliable travel estimates be accessed lawfully and consistently?
- What is the minimum feasible staff response workflow and what outcome evidence counts as “verified”?

10. References

[1] B. Das, D. Neupane, S. S. Gill, and G. B. Singh, “Factors affecting non-adherence to medical appointments among patients with hypertension at public health facilities in Punjab, India,” Journal of Clinical Hypertension, 2021. doi:10.1111/jch.14142.

[2] L. H. A. Salazar, W. D. Parreira, A. M. da Rocha Fernandes, and V. R. Q. Leithardt, “No-Show in Medical Appointments with Machine Learning Techniques: A Systematic Literature Review,” Information, vol. 13, no. 11, article 507, 2022. doi:10.3390/info13110507.

[3] AI Synergy Hackathon 2026, Participant Case File: “The Follow-Up Blind Spot” and “Round 1: Case Investigation Challenge.” Challenge-provided case scenario and simulated evidence pack.